

Medical Domain: Clinical Actionability

The submitted MDT plan is well structured, but conflicts with case-specific medication, timing, and handoff criteria.

1 Domain pressure

Clinical actionability

This case tests whether domain knowledge becomes a coordinated plan that meets case-level criteria.

Clinical decision dependencies

- Therapy

Balance active bleeding control with very-early post-DES thrombosis protection.
- Timing

Align anticoagulation restart with the benchmark-defined window.
- Handoff

State one unified escalation and discharge plan.

Representative domain risk

Medication continuation, restart timing, and discharge step-down are coupled decisions. Surface fluency alone does not establish case-level clinical actionability.

2 Medical failure chain

1

Conflicting clinical priorities
Upper GI bleeding conflicts with recent DES thrombosis protection.

2

Case-specific P2Y12 criterion is violated
The case rubric requires uninterrupted clopidogrel; the plan stops it before hemostasis.

3

Restart timing falls outside the case rubric
The plan uses day 5-7; the rubric specifies day 7-10.

4

Unified cath-lab decision is omitted
The plan does not restate that immediate cath-lab transfer is not the current priority.

5

Discharge step-down is omitted
The plan defaults to triple therapy rather than the discharge step-down required by the case rubric.

Fluent MDT structure can still fail case-specific safety criteria.

3 Representative case evidence

What looked complete

Five sections with staged medications; roles, escalation, and discharge drugs.

What failed

Four case-specific decisions conflict with or are absent from the submitted plan.

Artifact-level judge findings

- P2Y12

Case rubric:

clopidogrel uninterrupted

Submitted:

stopped before hemostasis
- Cath lab

Case condition:

state no immediate cath lab

Submitted:

not stated as unified decision
- DOAC

Case rubric:

day 7-10; not earlier than day 7

Submitted:

restart day 5-7
- Discharge

Case rubric:

step down from triple therapy

Submitted:

defaults to triple therapy

A structured answer can still fail artifact-level evaluation when medication timing or unified decisions conflict with case criteria.

StartupBench checks the submitted handoff against the case and benchmark-defined criteria.

Clinical grounding

Medication and escalation decisions are checked against the stated case and case rubric.

Workflow realism

The task requests a unified MDT plan for direct adoption in the medical record.

Safety constraints

Continuation, restart timing, and discharge sequencing are evaluated together.

Pattern: clinical actionability failure. The model produces a structured MDT plan, but key medication and timing decisions, together with the unified handoff, conflict with case-specific evaluation criteria.